Throat — Tonsils + Larynx

ENT | 3rd Year MBBS

Hi Sammy!
Throat = super
high yield in ENT!
Tonsils + Larynx!

Throat | Tonsillitis | Larynx | ENT

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Tonsillitis is asked
in every ENT exam!
Types + Rx = must know!

Acute vs Chronic Tonsillitis

Feature	Acute Tonsillitis	Chronic Tonsillitis
Etiology	Group A beta-hemolytic Strep	Recurrent acute attacks
Symptoms	Sore throat, fever, odynophagia	Recurrent sore throat, halitosis
Examination	Red, swollen tonsils +/- exudate	Tonsils may be small/fibrosed
Tonsils	Enlarged, congested	Cryptic, may express cheesy material
Jugulodigastric LN	Tender, enlarged	Non-tender, palpable
Treatment	Antibiotics (penicillin V)	Tonsillectomy if recurrent

Types of Chronic Tonsillitis

Type	Features
Chronic follicular	Yellow spots on tonsil surface (retained cheesy material)
Chronic parenchymatous	Enlarged tonsils (hypertrophic), common in children
Chronic fibroid	Small, fibrosed tonsils with crypts, common in adults

Grading of Tonsillar Enlargement (Brodsky)

Grade	Description
0	Tonsils within tonsillar fossa
1+	< 25% airway obstruction
2+	25-50% airway obstruction
3+	50-75% airway obstruction
4+	> 75% obstruction (kissing tonsils)

Complications of Acute Tonsillitis

- Local: Peritonsillar abscess (quinsy), parapharyngeal abscess, retropharyngeal abscess
- Systemic: Rheumatic fever, acute glomerulonephritis (post-streptococcal)
- General: Septicemia, otitis media, cervical abscess

Quinsy = peritonsillar
abscess = EMERGENCY!
Hot potato voice!

Peritonsillar Abscess (Quinsy)

Site:	Between tonsil capsule & superior constrictor muscle
Cause:	Complication of acute tonsillitis (Group A Strep)
Features:	Unilateral sore throat, trismus, hot potato voice, drooling
Exam:	Uvula deviated to opposite side, bulging soft palate
Treatment:	I&D (incision & drainage) + IV antibiotics + fluids
Interval:	Tonsillectomy after 6 weeks (or quinsy tonsillectomy)

Adenoids (Nasopharyngeal Tonsil)

Feature	Details
Definition	Hypertrophy of nasopharyngeal lymphoid tissue
Age	Most common in children (3-7 years), atrophy by puberty
Symptoms	Mouth breathing, snoring, nasal voice, recurrent URTI
Facies	Adenoid facies: open mouth, narrow palate, pinched nose
Complications	Secretory otitis media, recurrent sinusitis, OSA
Diagnosis	Lateral soft tissue XR nasopharynx, nasal endoscopy
Treatment	Adenoidectomy if symptomatic / causing complications

Tonsillectomy — Indications

1. Recurrent tonsillitis: 7 episodes/1yr, 5/yr x 2yrs, or 3/yr x 3yrs (Paradise criteria)
2. Peritonsillar abscess (after 2nd episode or with recurrent tonsillitis)
3. Obstructive sleep apnea (Grade 3-4 tonsillar hypertrophy)
4. Suspected malignancy (unilateral tonsillar enlargement)

Paradise Criteria!
7-5-3 Rule: 7 episodes in 1 year, OR 5/year for 2 years, OR 3/year for 3 years
= absolute indication for tonsillectomy! Most asked tonsillectomy indication question!

Recurrent laryngeal nerve = most important nerve in ENT surgery!

Acute vs Chronic Laryngitis

Feature	Acute Laryngitis	Chronic Laryngitis
Cause	Viral URTI, voice abuse	Smoking, GERD, chronic sinusitis
Symptoms	Hoarseness, cough, pain	Persistent hoarseness
Cords	Red, edematous	Thickened, polyps/nodules
Treatment	Voice rest, steam, antibiotics	Eliminate cause, speech therapy

Vocal Cord Paralysis (RLN Injury)

Feature	Unilateral Paralysis	Bilateral Paralysis
Voice	Hoarse, breathy	Near-normal (adducted cords)
Airway	Normal / mild dyspnea	Severe stridor / dyspnea
Cord position	Paramedian (cadaveric)	Median or paramedian
Common cause	Thyroid surgery, lung CA	Thyroid surgery (bilateral)
Treatment	Medialization thyroplasty	Tracheostomy + lateralization

Recurrent Laryngeal Nerve — Key Points

1. Left RLN loops under aortic arch; Right RLN loops under subclavian artery
2. Left RLN is longer -> more prone to injury / involvement by tumors
3. RLN supplies ALL intrinsic laryngeal muscles EXCEPT cricothyroid (ext. laryngeal n.)

★ Semon's Law!
Abductors (posterior cricoarytenoid) are affected FIRST in progressive RLN lesion.
So cord first goes to MEDIAN position (adducted), then later to CADAVERIC position.

Causes of Hoarseness (Viva Favorite)

Category	Causes
Inflammatory	Acute/chronic laryngitis, TB laryngitis, syphilis
Neoplastic	Laryngeal carcinoma, papilloma, vocal cord polyp
Neurological	Vocal cord paralysis (RLN palsy), myasthenia gravis
Functional	Vocal cord nodules (singer's), reinke's edema, puberphonia
Systemic	Hypothyroidism, rheumatoid arthritis (cricoarytenoid)

Tracheostomy — Indications + Types

Tracheostomy = the most important ENT surgical procedure!

Indications for Tracheostomy

Obstruction:	Laryngeal tumors, bilateral VC paralysis, foreign body
Prolonged intubation:	ICU patients ventilated > 10-14 days
Secretion management:	Head injury, tetanus, bulbar palsy
Pre-operative:	Before major head & neck cancer surgery
Sleep apnea:	Severe obstructive sleep apnea (last resort)

Types of Tracheostomy

Type	Details
Elective	Planned, done in OT under GA, between 2nd-4th tracheal rings
Emergency	Done in life-threatening situations, more complications
High	Above isthmus of thyroid (risk: subglottic stenosis)
Low	Below isthmus of thyroid (risk: innominate artery injury)
Percutaneous	Dilatational technique in ICU, Seldinger-guided
Minitracheostomy	Small tube for suctioning secretions only, not ventilation

Complications of Tracheostomy

EARLY

- Hemorrhage (most common intra-op)
- Pneumothorax / pneumomediastinum
- Subcutaneous emphysema
- Recurrent laryngeal nerve injury
- Posterior tracheal wall injury

LATE

- Tracheal stenosis (most common late)
- Tracheo-innominate artery fistula
- Tracheo-esophageal fistula
- Difficult decannulation
- Granulation tissue formation

Throat topics =
20+ marks guaranteed!
Go ace it Sammy!

Most Repeated LAQs (10–12 marks)

- 1. Classify tonsillitis. Describe acute tonsillitis & its complications
- 2. Indications for tonsillectomy. Describe dissection & snare method
- 3. Describe peritonsillar abscess — features, diagnosis & management
- 4. Causes of hoarseness. Describe vocal cord paralysis
- 5. Indications, types & complications of tracheostomy

Most Repeated SAQs (5–6 marks)

- 1. Paradise criteria for tonsillectomy
- 2. Adenoid facies — features & management
- 3. Semon's law & its clinical significance
- 4. Unilateral vs bilateral vocal cord paralysis
- 5. Complications of tracheostomy (early vs late)
- 6. Quinsy — clinical features & treatment

One-Liner Associations (MCQ/Viva Gold)

Question	Answer
Most common organism in acute tonsillitis	Group A beta-hemolytic Streptococcus
Hot potato voice + trismus + drooling	Peritonsillar abscess (quinsy)
Paradise criteria (7-5-3)	Indication for tonsillectomy
Adenoid facies	Open mouth, narrow palate, pinched nose
Most common cause of stridor in neonates	Laryngomalacia
Only laryngeal muscle supplied by SLN	Cricothyroid (external branch)
All intrinsic muscles supplied by	Recurrent laryngeal nerve (except CT)
Semon's law	Abductors affected first in RLN lesion
Singer's nodule	Vocal cord nodule (junction ant 1/3 & post 2/3)
Most common late complication of tracheostomy	Tracheal stenosis
Tracheostomy site	Between 2nd-4th tracheal rings
Grisel syndrome	Atlantoaxial subluxation post-adenoidectomy

♥ **Your voice matters, and so does every patient's voice, Sammy!**
Master the throat, and you'll speak volumes in your exams!